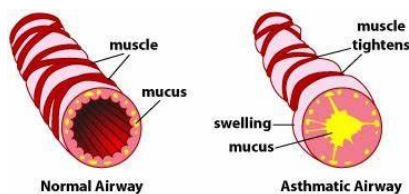


Asthma: What you need to know

Asthma Service, Chest Clinic

What is asthma?

Asthma is a common condition that affects the airways in and out of your lungs. These airways can become sensitive and inflamed, making it harder to breathe and causing **symptoms** such as **coughing, wheezing, shortness of breath, and chest tightness**.



Because the airways are so sensitive, certain things can trigger the muscles around the airways to tighten. This makes the airways narrower. The airway lining also becomes inflamed, causing the further narrowing of the airways and increased sputum

production.

Why do people get asthma?

There is no simple answer to this question. Whilst asthma often starts in childhood, it can develop at any stage in life. What we do know is that some things make asthma more likely:

- It can run in families – this is thought to be due to both genetics and the living environment you live and grow up in.
- Allergies make asthma more likely.
- Premature birth or low birth weight are thought to be risk factors, as are bronchiolitis and croup.
- Occupational exposure to certain substances such as dust or chemicals can lead to asthma in some individuals.
- Hormones can play a part – after puberty more girls than boys get asthma, and some women develop late-onset asthma during or after the menopause.
- Smoking, secondary exposure to smoke, and air pollution are also thought to be risk factors.



How is asthma diagnosed?

There is no one test to diagnose asthma. A diagnosis is made based on your symptom history and the results of breathing tests.

What triggers asthma symptoms?

Anything that irritates or inflames your airways can make your asthma worse. Common triggers include:

- Colds and viruses
- Allergies – e.g. animal hair, pollen
- Dust / Pollution / Smoke
- Stress or heightened emotions
- Temperature extremes / temperature change
- Hormones
- Strong smells – e.g. perfumes, aerosols, cleaning products

Some people can have exercise induced asthma but using a reliever medication before exercise can often prevent symptoms.

How is asthma treated?

Asthma is generally a lifelong condition. Whilst there is no cure for asthma, there are very effective treatments and most people who have asthma can be symptom free most of the time. The most common form of treatment is medication taken through an inhaler:

- **Preventer inhalers** try to stop asthma symptoms from occurring. They contain an inhaled steroid which helps to reduce symptoms by controlling the inflammation in the airways. Preventer inhalers need to be taken **every day** even if you are symptom free. The dose of steroid given is usually low and unlikely to cause any side effects. Occasionally the steroid can make your mouth sore but rinsing your mouth after use can reduce the chance of this.
- **Reliever inhalers** help to relieve asthma symptoms when they occur, so do not need to be taken routinely. You should take your reliever inhaler as soon as possible when you get symptoms such as wheeze, breathlessness, or chest tightness. Reliever inhalers contain medication called short-acting beta agonists (SABA) which work very quickly to relax the muscles surrounding the airways, making it easier to breathe.

There are several different inhaler devices. It is important to find one that you can use correctly, or you will not benefit fully from the medication. Your healthcare provider should assess your inhaler technique regularly.



If your asthma is not fully controlled by inhalers, there are **other treatments** which can be added in, so it is important to seek medical advice if this is the case.

Your asthma is considered **well controlled** if:

- You do not require your reliever inhaler more than 3 times per week
- Your asthma does not interfere with your normal daily activities
- You do not wake at night with asthma symptoms
- You haven't had a recent attack

Is asthma serious?

How serious asthma is varies from person to person. For most people, asthma can be well managed by using a preventer inhaler every day, and a reliever inhaler if symptoms flare up. However, around 5% of all people with asthma have a diagnosis of severe asthma which means that symptoms are more difficult to control even on a high level of treatment.



It is important to note that even if you consider your asthma to be mild, you can still experience a severe or life-threatening attack. The best way to reduce the risk is to take your asthma medications as prescribed, even if you feel well.

Following a **written asthma action plan** can help you to manage your asthma – ask your asthma nurse to provide you with a plan if you do not have one.

What is an asthma attack?

Asthma and Lung UK list the following as signs of an asthma attack:

- Your reliever isn't helping, or you are needing to take it more than every 4 hours
- Your symptoms (wheeze, cough, chest tightness, breathlessness) are getting worse
- You're breathless and finding it difficult to speak, eat or sleep
- Your breathing is getting faster, and you can't catch your breath properly

What to do:

- **Sit up straight** – don't lie down. Try to keep calm
- **Take one puff of your reliever inhaler** every 30-60 seconds up to a **maximum of 10 puffs**
- **If you feel worse at any point or don't feel better after 10 puffs, call 999 for an ambulance**
- **Repeat step 2 after 15 minutes** while you are waiting for an ambulance



Further Information:

Asthma and Lung UK website is an excellent source of useful, reliable, and up to date information.

<https://www.asthma.org.uk/>

Other formats

If you need this leaflet in another format such as Braille, large print, high contrast, British Sign Language or translated into another language, contact the Patient Advice and Liaison Service (PALS):

- 01752 439694
- Plh-tr.PALS@nhs.net

